

Day 18 of 42. Can we stop?

BED 7

Streptococcal aortic-valve endocarditis

Afebrile · CRP falling

ILLUSTRATIVE PATIENT

18

Six weeks comes mostly from 1950s observations

1950s

Lower-dose, single-drug
era

per POET-II investigators

2019

POET I: oral step-down

2026

POET-II: how long?

Only stable patients were randomized

- ✓ Afebrile for 2 days
- ✓ CRP < 50% of peak and < 40 mg/L
- ✓ WBC < $10 \times 10^9/\text{L}$
- ✓ Procalcitonin < 0.5 ng/mL
- ✓ No progression on TEE
- ✓ No further surgery planned

Randomized

508 stable patients:
stop now, or continue

255

TAILORED · STOP NOW

253

STANDARD · CONTINUE TO 4–6 WEEKS

OPEN-LABEL

13 CENTRES

MEAN AGE 70

25% WOMEN

STREPTOCOCCI 57%

S. AUREUS 24%

E. FAECALIS 19%

Tailored therapy cut antibiotics by 15 days

41

STANDARD · DAYS

-36.6%

15 DAYS FEWER



26

TAILORED · DAYS



Medians; 36.6% calculated. Illustrative calendar.

Bundgaard H, et al. NEJM 2026 · DOI 10.1056/NEJMoa2607887

13 more days alive without antibiotics

TAILORED

183

WITHIN SIX MONTHS



95% CI 12–13 · P < 0.001

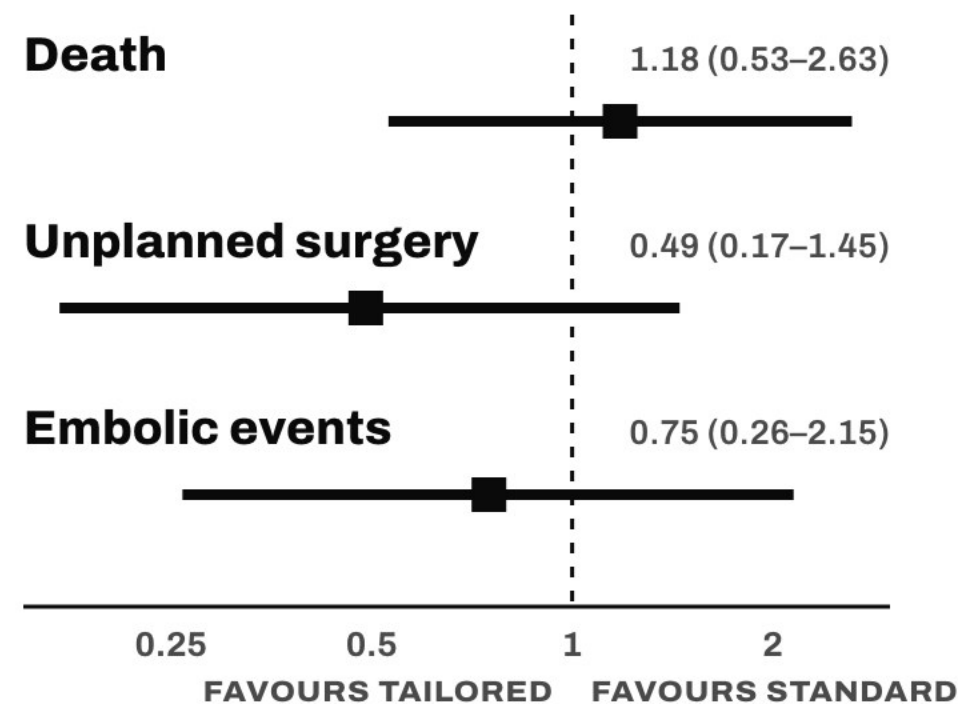
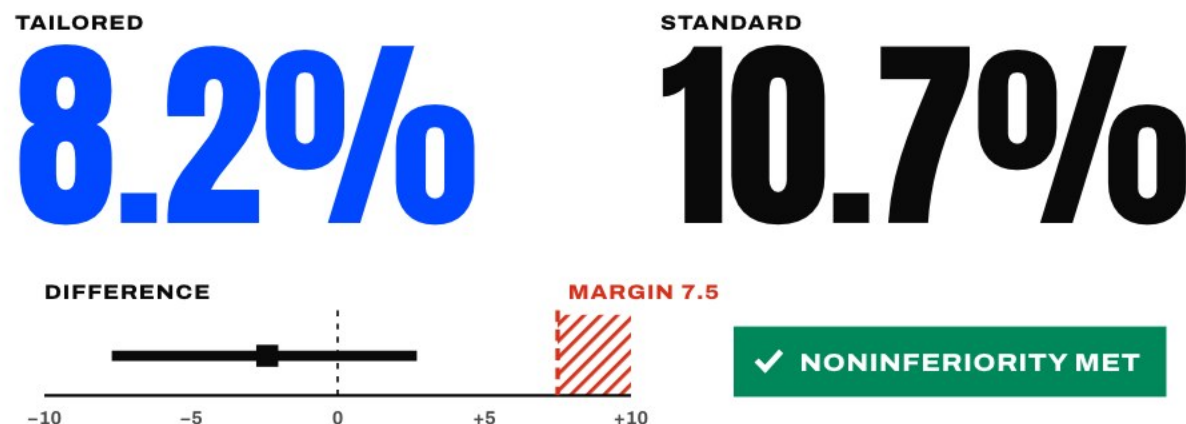
STANDARD

169



Medians

Safety: noninferior at six months



Wide intervals: few events

Relapse was more frequent: 5.1% vs 1.6%

TAILORED

13

PATIENTS RELAPSED

Mostly uncomplicated; antibiotics restarted.

STANDARD

4

P = 0.04

MEDIAN DAYS TO RELAPSE

28

74

E. faecalis relapsed most

RELAPSE BY PATHOGEN

E. faecalis



6.4%

S. aureus



3.3%

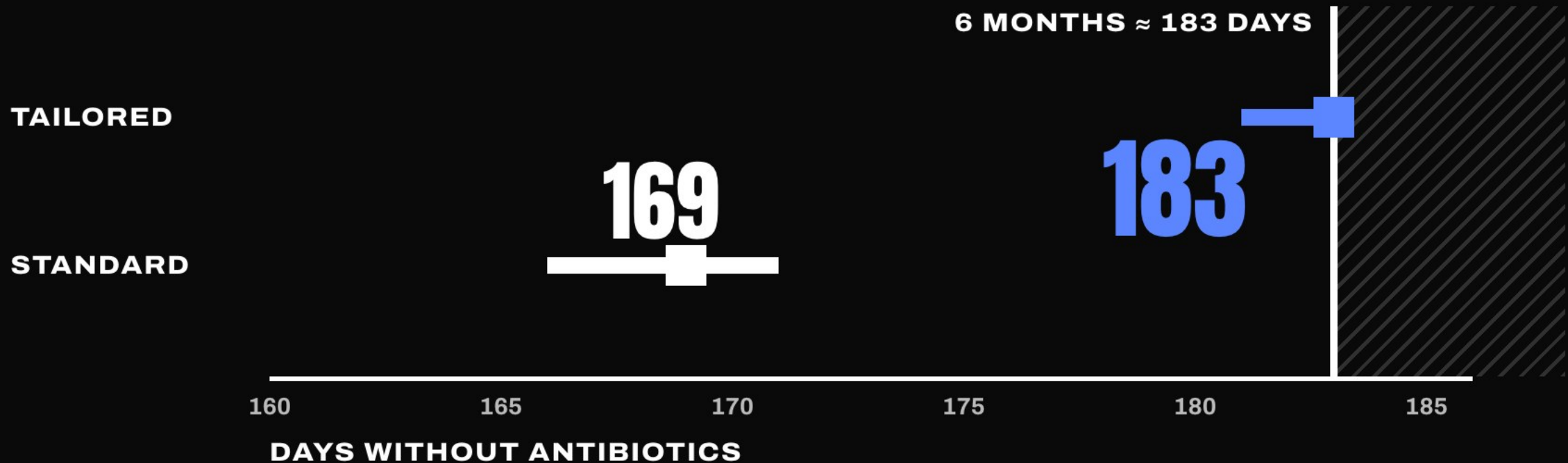
Streptococci



2.4%

Subgroup: hypothesis-generating

The primary endpoint is partly built in



■ Original primary (2020) included relapse

■ Real tests: safety, relapse. (my reading)

Five reasons for caution

-
- 1 Open-label**
 - 2 Selected stable patients — about half eligible**
 - 3 Specialized centres**
 - 4 Three pathogens only**
 - 5 Strict criteria, incl. TEE and procalcitonin**
-

My reading – not a guideline

BED 7

18

- ☒ All criteria met + strep or *S. aureus*: discuss stopping
- ☒ *E. faecalis*: be cautious
- ☐ Close follow-up in month one

Would you stop?

Bed 7 · all six criteria met · streptococcus

STOP

CONTINUE

What would change your mind?

Treat the response, not the calendar.

Questions?

Components of the safety endpoint

SIX MONTHS	TAILORED	STANDARD	HR (95% CI)
Death	5.1%	4.3%	1.18 (0.53–2.63)
Unplanned cardiac surgery	2%	4%	0.49 (0.17–1.45)
Symptomatic embolic events	2.4%	3.2%	0.75 (0.26–2.15)
Composite (primary safety)	21 (8.2%)	27 (10.7%)	Noninferiority met

Absolute difference –2.4 percentage points (95% CI –7.7 to 2.7); noninferiority margin 7.5 percentage points.

Composite, difference and margin: NEJM abstract. Components as reported by TCTMD, 28 Aug 2026.

Stabilization criteria – all required

Temperature < 38 °C (100.4 °F) for at least 2 days

CRP < 50% of peak and < 40 mg/L

WBC < $10 \times 10^9/L$

Procalcitonin < 0.5 ng/mL

No progression on TEE within 48 h

No further cardiac surgery planned

Before randomization: at least 2–4 weeks of antibiotics

2–4 weeks before randomization: NEJM abstract. Criteria as reported by TCTMD and ACC — full definitions in the NEJM supplement.

Pathogen subgroups

PATHOGEN	SHARE OF TRIAL	RELAPSE	DIRECTION
Streptococci	57.3%	2.4%	Favoured tailored
<i>S. aureus</i>	24.2%	3.3%	Favoured tailored
<i>E. faecalis</i>	18.5%	6.4%	Favoured standard

Consistent across native vs prosthetic valves and medical vs surgical treatment. Adding relapse to the safety endpoint, or excluding extracardiac infections: unchanged.

Subgroups are hypothesis-generating. As reported by TCTMD.

Registered vs published outcome

DESIGN PAPER 2020 · REGISTRY 2021		PUBLISHED 2026
Primary	One safety composite: death, unplanned surgery, embolism, relapse of bacteremia	Efficacy: days alive without antibiotics + safety: death, unplanned surgery, embolism
Relapse	Inside the primary composite	Key secondary: 5.1% vs 1.6%
Planned size	750 (registry, Dec 2021)	508 randomized

The change happened while the open-label trial was recruiting; when and why is not stated in the sources I could read. Authors report that adding relapse back to the safety endpoint left the result unchanged (TCTMD).

Østergaard L, et al. Am Heart J 2020;227:40–46 (PMID 32673830) · ClinicalTrials.gov NCT05144399 (duplicate record, 3 Dec 2021) · NCT03851575 (current).

ESC 2023 guideline – durations

Streptococci, native valve: 4 weeks (2 weeks with gentamicin in selected uncomplicated cases)

Streptococci, prosthetic valve: 6 weeks

***S. aureus* (MSSA): native valve 4–6 weeks; prosthetic at least 6 weeks**

***E. faecalis*: amoxicillin + ceftriaxone, 6 weeks**

Oral step-down after at least 10 days IV in stable patients: class IIa

Delgado V, et al. Eur Heart J 2023;44:3948–4042 (PMID 37622656). Read from an early-online copy — confirm wording in the final version.

POET I – the route question

Iversen K, et al. NEJM 2019;380:415–424 · PMID 30152252

Stable left-sided endocarditis

Partial oral step-down was noninferior to continued IV

POET I changed the route. POET-II changes the duration.

Background as cited in the ESC Congress 2026 press release.

Who is eligible?

About half of left-sided IE patients could be eligible (investigators' estimate)

The three pathogens cause about 70% of IE overall (per authors)

ESC discussant G. Habib: personalized duration — in selected patients, in specialized endocarditis centres

He asked for more detail on stabilization criteria, and caution in *E. faecalis*

ESC press release; TCTMD, 28 Aug 2026.

Funding and registration

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ClinicalTrials.gov NCT03851575

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ESC Congress 2026, Hot Line 1, Munich, 28 Aug 2026

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Bundgaard H, et al. N Engl J Med, published online 28 Aug 2026 (abstract, PubMed).